

LABORATORY MEDICINE PROGRAM

UUID: 9AD5008D-2288-4319-A686-7AB6FF840D8D

TCGA-EM-A3FO-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name:

MRN:

DOB:

Gender:

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #:

Collected:

Received:

Reported:

100-0-3

Specimen(s) Received

1. Parathyroid: RT.INFERIOR QS
2. Neck: Rt. central neck dissection
3. Thyroid: Total thyroid - stitch Lt. superior pole

Carcinoma, papillary, thyroid 8260/3
Site: thyroid, NOS C73.9 lw
11/22/11

Diagnosis

1. No pathological diagnosis: Parathyroid (right inferior) biopsy
2. No pathological diagnosis: Lymph nodes, 3 (right central neck) dissection specimen.
3. Angioinvasive classical variant papillary carcinoma, 4.4 cm, isthmus, and multifocal papillary carcinoma, bilateral: Thyroid, total thyroidectomy specimen. See Comment.

Comment

3. There is an abundant tumor burden of papillary carcinoma in this thyroid. The dominant 4.4 cm papillary carcinoma in the isthmus is a classical variant with focal tall cell, columnar cell, hobnail cell, and clear cell change (all minor components represent less than 10% of the tumor). The second dominant tumor, also in the isthmus, is a follicular variant papillary carcinoma 2.6 cm, and the third tumor, in the left lobe, is a follicular variant papillary carcinoma, 0.9 cm with invasive architecture. Vascular invasion in which intravascular tumor is seen with associated thrombus is identified for the dominant tumor in the isthmus.

Synoptic Data

Procedure:	Total thyroidectomy
Received:	Fresh
Specimen Integrity:	Intact
Specimen Size:	Right lobe:
	3.2 cm
	2.1 cm
	1.6 cm
	Left lobe:
	4.1 cm
	2 cm
	1.6 cm
	Isthmus +/- pyramidal lobe:
	6.4 cm
	4.4 cm
	3.3 cm

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
IPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed (Initials)	lw	11/22/11
Date Reviewed:	11/24/11	

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Specimen Weight: 69 g
Tumor Focality: Multifocal, bilateral
Multifocal, midline (isthmus)

----- DOMINANT TUMOR -----
Tumor Laterality: Isthmus
Tumor Size: Greatest dimension: 4.4cm
Additional dimension: 4.1 cm
Additional dimension: 3.3 cm
Histologic Type: Papillary carcinoma, classical (usual)
Papillary carcinoma, other variant: focal tall cell, hobnail cell, columnar cell and clear cell change, all less than 10% of tumor.
Classical (papillary) architecture
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Partially encapsulated
Tumor Capsular Invasion: Present, extent minimal
Lymph-Vascular Invasion: Present, focal extent (less than 4 vessels)
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified

----- SECOND TUMOR -----
Tumor Laterality: Isthmus
Tumor Size: Greatest dimension: 2.6cm
Histologic Type: Papillary carcinoma, follicular variant
Follicular architecture
Classical cytomorphology
Histologic Grade: Not applicable
Margins: Margins uninvolved by carcinoma
Tumor Capsule: Partially encapsulated
Tumor Capsular Invasion: Present, extent minimal
Lymph-Vascular Invasion: Indeterminate
Perineural Invasion: Not identified
Extrathyroidal Extension: Not identified
TNM Descriptors: m (multiple primary tumors)
Primary Tumor (pT): pT3: Tumor more than 4 cm, limited to thyroid or with minimal extrathyroidal extension (eg, extension to sternothyroid muscle or perithyroid soft tissues)
Regional Lymph Nodes (pN): pN0: No regional lymph node metastasis
Number of regional lymph nodes examined: 3
Number of regional lymph nodes involved: 0
Distant Metastasis (pM): Not applicable
Additional Pathologic Findings: Other: Multiple additional papillary microcarcinomas identified in the right lobe (0.1, 0.5cm) and the left lobe (0.1cm).

*Pathologic Staging is based on AJCC/UICC TNM,

Electronically verified by:

Gross Description

1. The specimen labelled with the patient's name and as "right inferior" consists of a piece of parathyroid tissue that measures 0.3 x 0.2 x 0.1 cm. It is frozen for intraoperative consultation.

1A frozen tissue resubmitted

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2. The specimen labeled with the patient's name and "right central neck dissection" consists of a piece of fibroadipose tissue that measures 3.5 x 3 x 0.6 cm. Multiple lymph nodes ranging from 0.2 to 0.5 cm are identified. Representative sections are submitted.

2A 2 nodes

3. The specimen labeled with the patient's name and as "total thyroid stitch left superior pole" consists of a thyroid gland that is oriented with sutures and weighs 69 g. The right lobe measures 3.2 cm SI x 2.1 cm ML x 1.6 cm AP, the left lobe measures 4.1 cm SI x 2 cm ML x 1.6 cm AP and the isthmus measures 6.4 cm SI x 4.4 cm ML x 3.3 cm AP. The external surfaces have fibrous adhesions. No parathyroid glands or lymph nodes are identified grossly. The external surface is painted with silver nitrate. The upper pole of the isthmus contains one partly friable well delineated solid nodule that measures 4.1 cm SI x 4.4 cm ML x 3.3 cm AP; the lower portion of the isthmus contains a separate well-circumscribed homogeneous tumor that measures 2.6 cm SI x 0.8 cm ML x 0.7 cm AP. The left lower lobe contains a well circumscribed solid tumor that measures 0.9 cm SI x 0.8 cm ML x 0.7 cm AP. The remainder of the thyroid tissue is somewhat nodular. Sections of isthmus nodules and normal tissue are stored frozen. Representative sections are submitted as follows:

3A-F right lobe, superior to inferior

3G-M isthmus upper nodule

3N-S isthmus lower nodule

3T-AA left lobe, superior to inferior

Quick Section Diagnosis

1. Right inferior parathyroid. qs: - Parathyroid tissue
OR informed